

1. Name of the medicinal product

SEDOXIL

2. Qualitative and quantitative composition

Mexazolam, 1 mg.

3. Pharmaceutical form

Uncoated tablets

4. Clinical information

4.1. Therapeutic indications

SEDOXIL is an anxiolytic agent of the benzodiazepines group.

SEDOXIL is indicated for the treatment of anxiety associated or not to psychoneurotic conditions.

Benzodiazepines are only indicated when the disorder is severe, incapacitating or causing great suffering and distress.

4.2 Posology and method of administration

Dosage of SEDOXIL should be customised to the needs of each patient taking into account the severity of the symptoms and the patient's age.

Adults: on average 1.0 to 3.0 mg daily, preferably divided into 3 intakes.

Elderly: do not exceed a 1.5 mg dose per day.

Children: SEDOXIL should not be administered to children.

The treatment of anxiety should be as short as possible. The patient should be monitored periodically and the need to continue treatment reassessed, especially if he/she shows signs of being asymptomatic. The duration of treatment should not exceed eight to twelve weeks including the tapering off period. In some cases, it may be necessary to extend treatment beyond the maximum recommended period. Should this be the case, extension should not occur without previous re-evaluation by a specialist.

In all cases, treatment should begin with the minimum recommended dose.

In case of missing, one or more doses do not take the dose if more than one hour has elapsed. Never double the dose.

At the onset of treatment, the patient should be supervised regularly, in order to reduce the dose or the frequency of administration in case it is necessary to prevent overdose through accumulation.

4.3 Contraindications

Myasthenia gravis, pregnancy, hypersensitivity to benzodiazepines, severe respiratory insufficiency, sleep apnoea syndrome and severe hepatic insufficiency.

4.4 Warnings and special precautions for use

Dependence

Use of benzodiazepines may lead to the development of physical and psychic dependence from these pharmaceutical products. The risk of dependence increases with dose and duration of treatment, and is greater in patients with a history of drug abuse or alcoholism.

Once physical dependence has been established, abrupt termination of the drug will be accompanied by withdrawal symptoms. These may consist of headaches, muscle pain, extreme anxiety, tension, restlessness, confusion and irritability. In severe cases the following symptoms may occur: derealisation, depersonalisation, hyperacusis, numbness and tingling of extremities, hypersensitivity to light, noise and physical contact, hallucinations or epileptic seizures.

Rebound of anxiety

A transient syndrome, whereby the symptoms that led to treatment with a benzodiazepine recur in an enhanced aggravated form, may occur on withdrawal of the treatment. It may be accompanied by other reactions including mood swings, anxiety or sleep disturbances and restlessness. Since the risk of rebound phenomena is greater after abrupt discontinuance of the drug, it is recommended that the dosage be decreased gradually.

Duration of treatment

The duration of treatment should be as short as possible (see Posology and method of administration), depending on the indication, but should not exceed 8 to 12 weeks in case of anxiety, including the tapering off process. Extension beyond this period should not take place without a reassessment of the situation. It may be useful to inform the patient when treatment is started that it will be of limited duration, and to explain precisely how the dosage will be progressively decreased. The patient should also be aware of the possibility of rebound phenomena, thereby minimising anxiety over such symptoms should they occur when implementing stoppage of the medicine.

When benzodiazepines with a long duration of action, such as mexazolam, are being used, it is important to warn against changing to a benzodiazepine with a short duration of action, as withdrawal symptoms might develop.

Amnesia

Benzodiazepines may induce anterograde amnesia. The condition occurs most often several hours after ingesting the product and therefore to reduce the risk patients would ensure that they would be able to have an uninterrupted sleep of 7-8 hours (see also undesirable side effects).

Psychiatric and paradoxical reactions

When using benzodiazepines reactions like restlessness, agitation, irritability, aggressiveness, delusions, rage, nightmares, hallucinations, psychoses, inappropriate behaviour and other related adverse behavioural effects are known to occur. If this is the case medication should be suspended. These reactions will more probably occur in children and the elderly.

Paradoxical reactions have occurred to schizophrenic patients with mexazolam

Specific patient groups

Elderly patients should be given a reduced dose (see dosage). A lower dosage is also recommended for patients with chronic respiratory insufficiency due to the risk of respiratory depression.

Benzodiazepines are not recommended for the primary treatment of psychotic illness.

Benzodiazepines should not be used alone to treat depression or anxiety associated with depression (suicide might be precipitated in such patients).

Benzodiazepines should be used with extreme caution in patients with a history of drug abuse and alcoholism.

Mexazolam should be used with extreme caution in patients with cardiac, renal or hepatic disorders and in patients with encephalic organic disorders.

4.5 Medicinal interactions and other

Not recommended: concomitant intake with alcohol

The sedative effect may be enhanced when mexazolam is used in combination with alcohol; this affects the ability to drive or use machines.

Take special precautions: when associated with CNS (Central Nervous System) depressants

Enhancement of the central depressive effect may occur in cases of concomitant use of Mexazolam with antipsychotic (neuroleptics), hypnotics, anxiolytics/sedatives, antidepressant agents, narcotic analgesics, anti-epileptic products, anaesthetics, and antihistamine sedatives.

In the case of narcotic analgesics enhancement of euphoria may also occur leading to an increase in physical dependence.

In experimentation animals, the following interactions were reported:

Drugs enhancing mexazolam effects: chlorpromazine, haloperidol, diazepam, cyproheptadine, aminopyrine, phenobarbital, ethanol, sulpiride, trichlormethiazide and alpha-methyldopa.

Drugs antagonising mexazolam effects: imipramine, amitriptyline and chlorpheniramine.

Drugs, which have not shown interaction with mexazolam: diphenylhydantoin, scopolamine Butylbromide, gerfanate, propranolol and pindolol.

4.6 Use during pregnancy and lactation

Although clinical trials in animals did not suggest any harmful effects of mexazolam during pregnancy, its safety in humans was not established during pregnancy.

Benzodiazepines may cause congenital abnormalities when administered during the first trimester of pregnancy. If for compelling medical reasons mexazolam is administered during the late phase of pregnancy or during labour at high doses, effects on the neonate such as hypothermia, hypotonia and moderate respiratory depression can be expected.

Infants born to mothers who took benzodiazepines chronically during pregnancy may have developed physical dependence and may be at some risk for developing withdrawal symptoms in the postnatal period.

Since mexazolam may be found in the breast milk, it should not be given to breast feeding mothers.

Every woman of childbearing potential should contact her physician if she intends to become or suspects that she is pregnant.

4.7 Effects on the ability to drive or use machines

Sedation, amnesia, impaired concentration and impaired muscular function resulting from the administration of SEDOXIL may adversely affect the ability to drive or use machines. If insufficient sleep duration occurs, the likelihood of impaired alertness may be increased (see also interactions).

4.8 Undesirable side effects

The undesirable side effects of benzodiazepines are, most of the time, an extension of the pharmacological actions and include drowsiness during the day, numbed emotions, reduced alertness, confusion, fatigue, headache, light headedness, muscle weakness, ataxia or double vision.

Rarely (<0.1%) the following adverse events were reported with mexazolam: altered tongue movements, hypotension, light headedness, nausea, vomiting, anorexia, gastric discomfort, gastric pain, abdominal pain, diarrhoea, hypersensitivity reactions (e.g. skin reactions), decreased libido, increase of alkaline phosphatase, anaemia and leukopenia. Infrequently (0.1 - 5%) were reported drowsiness, vertigo, dizziness, headache, ataxia, dry mouth, weakness, increase of aspartate aminotransferase, alanine aminotransferase and gamma-glutamyl-transferase.

Undesirable side effects occur predominantly at the start of therapy and usually disappear upon continued administration of the medication.

Amnesia

Anterograde amnesia may occur when using therapeutic doses of benzodiazepines, the risk increasing when higher doses are used. Amnestic effects may be associated with inappropriate behaviour (see Warnings and special precautions for use).

Depression

Pre-existing depression may arise when benzodiazepines are used.

Psychiatric and paradoxical reactions

Reactions like restlessness, agitation, irritability, aggressiveness, delusions, rage, nightmares, hallucinations, psychoses, inappropriate behaviour and other related adverse behavioural effects are known to occur and become particularly severe with benzodiazepines. They are more likely to occur in children and the elderly.

Dependence

Even at therapeutic doses, SEDOXIL may lead to the development of physical dependence: discontinuance of the therapy may result in withdrawal or rebound phenomena (see Warnings and special precautions for use). Psychic dependence may also occur. Abuse of benzodiazepines has also been reported.

4.9 Symptoms and treatment of overdose

As with other benzodiazepines, a SEDOXIL overdose should not present a threat to life, unless combined with other CNS (Central Nervous System) depressants, including alcohol.

In the management of an overdose of any pharmaceutical product, it should be borne in mind that multiple agents might have been taken.

Following an overdose with SEDOXIL vomiting should be induced (within one hour) if the patient is conscious or if the patient is unconscious gastric lavage should be undertaken (with an endotracheal tube in place, to prevent aspiration of vomit). If there is no advantage in emptying the stomach, activated charcoal should be given to reduce SEDOXIL absorption. Special attention should be paid to respiratory and cardiac functions and the patient should be closely observed in an intensive care unit.

Overdose of benzodiazepines is usually manifested by central nervous system depression, ranging from drowsiness to coma. In mild cases, symptoms include drowsiness, mental confusion and lethargy, and in more serious cases, symptoms include ataxia, hypotonia, hypotension, respiratory depression, rarely coma and very rarely death.

Flumazenil (a specific benzodiazepine receptor antagonist) may be useful as an antidote.

5. Pharmacological properties

5.1. Pharmacodynamic properties

Pharmacotherapeutic Classification: II – 8- a – Anxiolytics, sedatives and hypnotics

ATC Classification: N05 BA

SEDUXIL is a benzodiazepine. As a rule, benzodiazepines act as central nervous system depressants producing all levels of depression from light sedation to hypnosis and coma, depending on the dosage.

Locations and operating mechanisms are not completely defined. It has been established that benzodiazepines influence the neurotransmission mediated by gamma aminobutyric acid (GABA), through the stimulation of specific receptors. The connection to these receptors may be modulated either by GABA either by the chlorine ion.

Sedative effect – testing in mice, rats, hamsters and monkeys demonstrated that mexazolam induces an increased inhibition of conflict behaviour, violence, aggression and excitement than diazepam. Studies in cats and rabbits suggest these sedative effects may result from the intervening action of the pharmaceutical product on the limbic system, including the amygdala and the hypothalamus.

Anticonvulsant action – testing in mice, rats and monkeys demonstrated that mexazolam induces a higher inhibiting effect of the seizures induced by bemegride and pentetrazol than diazepam.

Muscular relaxing effects - mexazolam inhibited decerebrate rigidity and lowered the activity of the gamma motor neurons in cats.

Influence on motor function control – testing on mice, rats and dogs demonstrated that the influence of mexazolam on motor function (e.g. inhibition of spontaneous motor activity, muscle relaxant effects, inhibitory effect on the standing reflex and ataxic effect), is limited.

5.2 Pharmacokinetic properties

After the oral intake of mexazolam, the unchanged pharmaceutical product goes undetected in the bloodstream, only its active metabolites CND (chloro-nordazepam) and COX (chloro-oxazepam). It is assumed that mexazolam is hydroxylated and conjugated in the liver through two metabolic pathways, one of the benzodiazepine type (active metabolite) and the other of the benzophenone type (inactive metabolites).

CND is the main plasma metabolite of mexazolam. With a single dose of mexazolam, the peak plasma concentration of CND is reached 1 to 2 hours after administration. The apparent volume of distribution is 150 L. The applicable pharmacokinetic model is of the bicompartamental type. After repeated administration, the half-life of CND and COX is rather long (130 to 200 hours).

Protein binding is > 90% for both CND and COX.

Less than 10% of the orally administered mexazolam dose is discarded as urine metabolites thus the majority of the medicine is excreted through bile. COX makes up > 50% of the total metabolites excreted, being mainly in conjugated form.

5.3 Preclinical safety data

Studies with mexazolam were carried out on several animal species including mice, rats, rabbits, dogs and monkeys that revealed a wide safety margin and an extremely reduced toxic capacity. As a matter of fact the LD50 (median Lethal Dose) for oral intake was 4.687 mg/kg in mice, 8100 mg/kg in Wistar-Imamichi rats and 1400 mg/kg in Fischer rats.

In sub acute toxicity studies in rats (10, 30, 125, 250, 500 and 1500 mg/kg, oral intake, 5 weeks) mortality only occurred in the group that received 1500 mg/kg. Hypertrophy of the hepatocytes occurred with doses equal to or in excess of 250 mg/kg, but disappeared after the discontinuance of the medicine. Administration of mexazolam, through oral intake, during 26 weeks, in rats that received daily doses of 2, 10, 50 or 125 mg/kg revealed that the weight of the liver, kidneys and suprarenal glands increased in the females of the group that received the highest dose. However, the histologic exam did not reveal any meaningful alterations.

Alterations were not observed in terms of mating or fertility in male rats (2, 10 or 20 mg/kg during 9 weeks) neither lethal nor teratogenic effects in the foetuses of female rats (2, 10 or 20 mg/kg, from the 14th day pre-mating until the 7th day of gestation). The administration of mexazolam during the organogenesis period (rat - 2, 10 or 20 mg/kg/day during 11 days; rabbit – 0.04, 0.2, 1 or 5 mg/kg/day during 13 days) did not cause death or teratogenic effects in the embryos and foetuses. The administration of mexazolam during the perinatal and nursing periods (rats - 2, 10 or 10 mg/kg/day during 4 weeks from the 17th day of gestation) did not generate any alterations in the delivery, in the nursing neither in the offspring (external anomalies, post-natal differentiation time and procreating capacity). Mexazolam did not display any in vitro mutagenic potential, in the reversion test and in the Rec assay test. The antigenicity of mexazolam was researched in guinea pigs and rabbits and no evidence was found of exerting any antigenic influence.

6. Pharmaceutical information

6.1. List of excipients

Mannitol, maize starch, hydroxypropyl cellulose, sodium lauryl sulphate and calcium stearate.

6.2 Incompatibilities

Not known.

6.3 Shelf life

3 years.

6.4. Special storage precautions

This product should be kept out of the sight and reach of children.

The product should not be used after the expiry date which is stated on the carton after EXP. The expiry date refers to the last day of that month.

The product should be stored in the original package in order to protect from light.
Store below 30°C

6.5 Type and contents of the container

Sedoxil 1 mg tablets are white, circular and biconvex, engraved BIAL on one side and SD/1 on the other.

The tablets are packed in Alu/Alu blisters containing 30 tablets.

6.6. Instructions for use and handling

Does not require any special instructions for use or handling.

7. Marketing Authorization Holder

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8. Marketing Authorization Number

9. Date of Marketing Authorization Renewal

10. Date of revision (partial) of text

April, 2019